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LEARNER GUIDE

For

Improve Employee Wellness and Work Resilience

Course Code: C1158

Conducted by
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DOCUMENT VERSION CONTROL RECORD

Version Number	Effective Date of Release	Summary of Included Changes	Author
1.0	2 October 2026	One-day course courseware with ten real-use-case activity packs and detailed learner procedures.	Dr. Alfred Ang

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How to Use This Guide

Read the concept section before each activity, then complete the activity in its individual folder. Detailed procedures belong here and in the activity Markdown files; the slide deck remains visual and concept-led.

Evidence boundary: Each activity uses a documented real event as context. All organisation details, metrics and operational inputs not in the cited source are explicitly fictional training assumptions.

Course Learning Outcomes

- LO1: Identify workplace health and wellness trends, technologies and research implications for employees and the organisation.
- LO2: Implement inclusive health and wellness programmes for different employee groups and translate policies into day-to-day practices.
- LO3: Communicate programme benefits through appropriate channels and assess effectiveness using employee feedback and programme evidence.

The Real-Event Activity Portfolio

Activities 1–10 use published research and current Singapore guidance from PDPC, TAFEP and HPB as real-world anchors. Every activity is self-contained and distinguishes sourced evidence from simulated workplace data.

Activity 1 — Privacy-Safe Wearable Wellness Pilot: primary case source

Activity 2 — Segmented Workforce Wellbeing Needs Assessment: primary case source

Activity 3 — Evidence and Technology Selection Board: primary case source

Activity 4 — Resilience Ecosystem After a Service Surge: primary case source

Activity 5 — Whole-Person Wellness Programme Portfolio: primary case source

Activity 6 — Mental-Wellbeing Policy-to-Practice Playbook: primary case source

Activity 7 — Participation and Partnership Recovery Plan: primary case source

Activity 8 — Inclusive Multi-Channel Wellbeing Campaign: primary case source

Activity 9 — Employee Feedback and Listening System: primary case source

Activity 10 — Wellbeing Programme Management Review: primary case source

Topic 1 — Workplace Health, Wellness and Evidence

Focus: wellbeing dimensions · Singapore requirements · needs · trends · technology · evidence.

Key Concepts

- Health: A state and capability shaped by physical, mental and social conditions—not merely the absence of illness.
- Wellness: Intentional practices and enabling conditions that support people to function and thrive.
- Evidence: A useful decision separates verified facts, workforce data, professional judgement and assumptions.
- System: Job design, managers, culture, access and individual resources interact; a perk alone is not a strategy.

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Wellbeing is an operating condition, not a collection of perks.

Key point: Wellbeing is an operating condition, not a collection of perks. Design work and support so that people can be healthy, contribute and recover sustainably.

Health and wellness: related, not interchangeable

Perspective A

- Health includes physical, mental and social functioning and limitations.
- It may require clinical care, reasonable support and risk controls.
- Employers should not diagnose or infer a condition from behaviour.

Perspective B

- Wellness describes choices, resources and conditions that support functioning.
- Participation should be voluntary, inclusive and free from coercion.
- Programme design complements—not replaces—safe work and medical care.

Interdependent dimensions of wellbeing

- Career: Meaning, manageable expectations, strengths and development.
- Social: Belonging, support, respect and constructive relationships.
- Financial: Security, capability and access to trustworthy guidance.
- Physical: Energy, movement, rest, nutrition and safe work.
- Community: Connection, contribution and confidence in the wider environment.
- Mental and emotional: Psychological safety, recovery, coping resources and access to help.

From wellbeing conditions to organisational outcomes

1. Job demands and resources
2. Daily experience
3. Energy and health behaviours
4. Attendance and engagement
5. Performance and retention
6. Customer and business outcomes

Population health indicators and workplace decisions

Perspective A

- Life expectancy estimates years lived; health-adjusted life expectancy also considers years lived in less than full health.
- Population indicators can signal prevention priorities and health inequality.
- They do not diagnose a workforce or measure one employer programme.

Perspective B

- Use workforce needs, job risks, access and employee voice for workplace decisions.
- Do not convert population averages into individual targets or employment judgements.
- Connect prevention with safe work, timely support and public-health pathways.

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Singapore workplace requirements: the practical baseline

- PDPA: Use a lawful purpose, notify appropriately, protect data, limit retention and enable applicable access/correction rights.
- Workplace safety and health: Manage workplace risks, including work conditions that can harm physical or psychological health.
- Employment practices: Make objective decisions, maintain confidentiality and provide fair, respectful support.
- Future fairness regime: Prepare for Singapore's Workplace Fairness Act implementation; do not describe future duties as already in force.

Singapore health and employment touchpoints

- Employment Act sick leave: Eligible employees have statutory outpatient and hospitalisation leave subject to service, notification and certification rules.
- Healthier SG: A national preventive-health initiative centred on family doctors, health plans and community support; employers can signpost without accessing clinical data.
- HPB workplace support: Current workplace programmes and tools can complement an organisation's needs-led plan; validate eligibility and terms before commitment.
- Employment data boundary: Administer leave and benefits with necessary information; do not infer a blanket right to employee diagnoses or unrelated health data.

Privacy-by-design for wellbeing data

1. Define a necessary purpose
2. Collect the minimum
3. Separate identifiers
4. Restrict access
5. Set retention and deletion
6. Report only safe aggregates

Consent and power imbalance

Perspective A

- A checkbox does not make every use fair or necessary.
- Employees may feel pressure when managers sponsor participation.
- Avoid linking sensitive health disclosure to performance decisions.

Perspective B

- Offer a genuine choice and a useful non-participation path.
- Explain purpose, recipients, retention and withdrawal clearly.
- Use independent providers and aggregates where practical.

Data risks in wearables and wellness platforms

- Function creep: Step or sleep data becomes input for unrelated employment decisions.
- Re-identification: Small teams or rare conditions reveal identities despite aggregation.
- Vendor exposure: Cloud, subcontractor and cross-border processing expands the risk surface.
- Exclusion: Device, language, disability or algorithm design disadvantages some groups.

Factors that shape employee health

- Work design: Demand, control, staffing, schedules, role clarity and workload variability.

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- Relationships: Manager behaviour, team norms, bullying, support and psychological safety.
- Personal context: Health, caring duties, finances, life stage and access to services.
- Environment: Ergonomics, noise, heat, travel, food options and digital load.

Job Demands–Resources lens

1. Identify demands
2. Identify protective resources
3. Find imbalance
4. Redesign work
5. Add targeted support
6. Monitor recovery and outcomes

Mental-health continuum: respond without diagnosing

- Thriving: Sustainable energy, connection and effective functioning.
- Struggling: Observable changes in work, mood, energy or connection may need a supportive check-in.
- Unwell: Persistent impairment may need professional support and workplace adjustment.
- Crisis: Immediate safety concerns require the organisation's emergency and escalation protocol.

Supportive observation versus diagnosis

Perspective A

- Describe observable changes: missed deadlines, withdrawal or unusual fatigue.
- Ask privately, listen, clarify work needs and signpost support.
- Escalate immediate risk using the approved protocol.

Perspective B

- Do not label anxiety, depression, bipolar disorder, OCD, psychosis, eating disorders or substance misuse.
- Do not promise absolute secrecy when safety action may be necessary.
- Do not collect clinical details that the work decision does not require.

Needs-assessment evidence

- Workforce voice: Anonymous survey, focus groups, interviews and employee-resource groups.
- Operational data: Absence, overtime, turnover, incidents, workload and schedule patterns.
- Service data: Aggregated EAP or benefit utilisation and access barriers.
- Risk evidence: WSH assessments, ergonomic observations and job-demand reviews.

Responsible segmentation

1. Choose a decision need
2. Use relevant groups
3. Set minimum cell sizes
4. Compare access and outcomes
5. Avoid stereotyping
6. Co-design targeted support

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Workplace wellbeing trends

- Whole-person design: Connect physical, mental, social, financial and career support.
- Hybrid access: Blend onsite, remote, synchronous and self-directed pathways.
- Personalisation: Offer choice while protecting privacy and accessibility.
- System accountability: Measure manager and job-design conditions, not only individual habits.

Technology options and trade-offs

- Wearables: Continuous feedback can motivate, but creates surveillance and accuracy risks.
- Telehealth: Improves reach, but depends on clinical quality, privacy and digital access.
- Apps and gamification: Can support habits; novelty, competition and accessibility may limit benefit.
- AI coaching: Scales prompts and navigation; requires safety boundaries, bias checks and human escalation.

Evidence-to-decision pipeline

1. Frame the problem
2. Search credible sources
3. Appraise relevance and quality
4. Compare with workforce evidence
5. Pilot with safeguards
6. Decide, monitor and adapt

A fashionable tool is not a wellbeing outcome.

Key point: A fashionable tool is not a wellbeing outcome. Choose technology only when a defined need, safeguard and measure justify it.

Activity 1 — Privacy-Safe Wearable Wellness Pilot

Outcome: LO1. Apply legislative and ethical requirements to a workplace wellness technology decision.

Goal and Deliverable

Assess whether a voluntary wearable pilot can address a defined employee need without creating surveillance, exclusion or unsafe data use.

- Deliverable: One-page pilot decision, annotated data-flow map and privacy/equity risk register

Real-World Scenario

Singapore's PDPA sets obligations including accountability, notification, consent, purpose limitation, accuracy, protection, retention limitation, transfer limitation and applicable access/correction rights. These obligations shape the design of employer-sponsored wellbeing data flows.

MarinaLink Logistics, a fictional Singapore employer with 640 office and shift employees, is considering voluntary wearables after staff reported fatigue and low movement. The vendor proposes individual dashboards for managers and a team leaderboard.

Source: https://www.pdpc.gov.sg/overview-of-pdpa/the-legislation/personal-data-protection-act/data-protection-obligations

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Training Assumptions and Guardrails

- MarinaLink Logistics, all workforce figures and vendor terms are fictional training inputs.
- The proposed device records steps, heart rate, sleep estimates, location during challenges and a user identifier.
- Participation is advertised as voluntary, but line managers will distribute devices and see individual dashboards under the draft plan.
- Thirty per cent of shift staff share or do not regularly use a compatible smartphone; some employees cannot safely pursue step targets.

Scenario Questions

Question 1: What decision purpose is necessary and proportionate, and which proposed fields are excessive?

Question 2: Where could power imbalance undermine voluntariness or trust?

Question 3: Which privacy, accessibility and fairness controls are pilot gates?

Question 4: Should the pilot proceed, proceed conditionally or stop—and what evidence supports the decision?

Detailed Step-by-Step Activity

Step 1 — Frame the decision

Write one measurable employee need and one pilot decision. Exclude performance management, diagnosis and productivity surveillance from scope.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Map the data flow

Map collection, device/app processing, vendor storage, employer access, reporting, transfer, retention and deletion. Name the responsible party at every handoff.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Classify necessity

For each data field, record purpose, necessity, sensitivity and the least-intrusive alternative. Remove any field without a defensible need.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Test voluntariness

Examine recruitment, incentives, manager involvement and the non-participant experience. Redesign the opt-in so refusal has no employment disadvantage.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Assess equity

Use the employee personas to identify device, disability, language, shift and health barriers. Provide equivalent non-device participation pathways.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 6 — Design controls

Specify notice, consent where applicable, access roles, aggregation threshold, security, vendor terms, retention, deletion, breach response and complaint route.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Set pilot gates

Define readiness, participation, trust, access, incident and outcome measures. Add a stop trigger for privacy, safety or inequity concerns.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Make the decision

Recommend proceed, conditional proceed or stop. State residual risks, accountable owner, review date and evidence needed before scale.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: The submission defines a necessary purpose, minimises fields, removes manager access to individual data, offers a genuine alternative, specifies reporting thresholds and assigns every residual risk to an owner and gate.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 2 — Segmented Workforce Wellbeing Needs Assessment

Outcome: LO2. Identify the current health and wellness state of different employee groups using privacy-safe evidence.

Goal and Deliverable

Combine workforce voice and operational signals to prioritise needs without diagnosing employees or exposing small groups.

- Deliverable: Needs-assessment matrix and a two-priority management brief

Real-World Scenario

Gallup frames wellbeing through career, social, financial, physical and community elements and links the employee experience to outcomes such as engagement, absence and retention. The framework is a lens for inquiry, not a diagnostic instrument.

NorthQuay Services, a fictional 420-person firm, has a head office, a 24/7 contact centre and mobile technicians. Leaders want one annual wellness event, while employee feedback suggests different barriers by group.

Source: https://www.gallup.com/workplace/404105/importance-of-employee-wellbeing.aspx

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Training Assumptions and Guardrails

- All survey, absence, overtime, turnover and focus-group data in the activity pack are fictional.
- Groups with fewer than ten responses must not be reported separately.
- Operational indicators have multiple possible causes and cannot diagnose health conditions.
- The training dataset intentionally includes missing responses and unequal response rates.

Scenario Questions

Question 1: What can each evidence source support, and what can it not support?

Question 2: Which needs are shared and which differ by workforce group?

Question 3: Where could response bias, small numbers or stereotyping distort the result?

Question 4: Which two priorities should management address first, and why?

Detailed Step-by-Step Activity

Step 1 — Define the management decision

State the programme decision, time horizon and employee groups. Do not begin by choosing an intervention.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Create an evidence inventory

List survey, focus group, operational, EAP and risk data. Record source, period, coverage, privacy risk and known limitation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Check data quality

Calculate response and missing-data rates. Flag small cells, inconsistent denominators, outliers and groups that are not adequately represented.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Describe each segment

Summarise wellbeing conditions and barriers for office, contact-centre and mobile employees using neutral, non-diagnostic language.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Triangulate

Require at least two independent signals before treating a need as established. Record contradictions rather than hiding them.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Apply an equity lens

Compare access, job demands, control, schedule, manager support and service reach. Avoid attributing group differences to personal weakness.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 7 — Prioritise transparently

Score materiality, employee voice, modifiability, equity, urgency and evidence confidence. Select two priorities and one research gap.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Prepare the management brief

Present the current-state matrix, limitations, priority rationale, next evidence action and privacy-safe reporting rules.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: Every priority uses at least two evidence signals, small groups are suppressed, uncertainty is explicit, no condition is diagnosed, and recommended action addresses a workplace factor plus an access need.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 3 — Evidence and Technology Selection Board

Outcome: LO1. Identify and appraise wellbeing trends, technologies and research for workplace relevance.

Goal and Deliverable

Compare wearables, telehealth, wellbeing apps, gamification and AI coaching against evidence, need, safety, inclusion and implementation fit.

- Deliverable: Source appraisal ledger, weighted option board and pilot hypothesis

Real-World Scenario

The NCBI review describes employee wellness programmes as employer-sponsored efforts to support healthy behaviour and reduce health risk, and stresses leadership, alignment, scope, accessibility, partnerships and communication. It also notes participation barriers such as awareness, time, manager support, perceived benefit, access and privacy.

BrightForge Engineering, a fictional manufacturer, has S$80,000 for one pilot. Vendors propose a wearable challenge, telehealth access, a mindfulness app with gamification, or AI coaching. The workforce includes production shifts, engineers and mature technicians.

Source: https://www.ncbi.nlm.nih.gov/books/NBK201567/

Training Assumptions and Guardrails

- Vendor claims, prices and implementation times are fictional and have not been independently validated.
- The workforce need is access to timely support and recovery, not increased monitoring.
- Mobile access and English literacy vary across groups.

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- The board may reject all four options or select a non-technology response.

Scenario Questions

Question 1: Which source claims are credible, relevant and transferable to this workforce?

Question 2: What employee need does each option address—and what new risk does it create?

Question 3: Which criteria and weights should govern the choice?

Question 4: What pilot result would justify scale, adaptation or termination?

Detailed Step-by-Step Activity

Step 1 — Frame a PICO-style question

Define population, intervention, comparator and outcome for the workplace decision. State acceptable implementation time and risk boundaries.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Build a source ledger

Record source type, date, population, intervention, outcome, limitations and conflicts of interest. Separate research from vendor assertion.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Appraise transferability

Compare research context with workforce, setting, access, culture and baseline need. Rate direct, partial or weak relevance with reasons.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Map option logic

For each technology, show need, mechanism, expected behaviour or condition change, outcome and possible harm.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Choose decision criteria

Weight need-fit, evidence quality, privacy, safety, inclusion, usability, vendor capability, cost and evaluability before scoring options.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Run the option board

Score each option independently, compare rationales, resolve material disagreements and record sensitivity to changed weights.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Define the pilot

Write a testable hypothesis, target group, comparison or baseline, safeguards, reach and outcome measures, duration and stop triggers.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 8 — Issue a decision note

Recommend one option, a combined/non-technology alternative or no pilot. State evidence gaps and due-diligence actions.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: The recommendation distinguishes research from vendor claims, maps each option to a defined need, includes privacy and accessibility gates, tests weight sensitivity and states scale/adapt/stop criteria.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Topic 2 — Programme Design and Implementation

Focus: programme portfolio · inclusion · policy-to-practice · resilience · participation · governance.

Key Concepts

- Portfolio: Combine prevention, protection, early help, support and recovery rather than relying on one programme.
- Fit: Design for employee needs, operating context, access barriers and cultural expectations.
- Implementation: Turn policy into roles, routines, resources, escalation and evidence.
- Resilience: Build individual, team and organisational capacity while reducing preventable demands.

Start with the work, then add support.

Key point: Start with the work, then add support. A resilience workshop cannot compensate for chronic overload, unclear priorities or unsafe management behaviour.

Programme portfolio

- Prevention: Healthy work design, risk reduction, ergonomics, financial capability and connection.
- Promotion: Movement, nutrition, sleep, learning and inclusive social participation.
- Early help: Manager conversations, screening boundaries, navigation and EAP access.
- Treatment and recovery: Benefits, clinical pathways, adjustments and return-to-work support.

Common programme types

- Physical wellbeing: Activity, ergonomics, preventive checks, nutrition and chronic-condition support.
- Mental wellbeing: Psychological safety, EAP, counselling, peer support and manager capability.
- Financial wellbeing: Budgeting, debt support, benefits navigation and retirement planning.
- Social and community: Connection, volunteering, inclusion and team practices.

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Disease management and prevention

Perspective A

- Disease-management support serves employees with an identified condition.
- It may include navigation, medication adherence and work adjustment.
- Clinical decisions belong with qualified professionals.

Perspective B

- Prevention reduces risk and strengthens protective conditions before harm.
- It includes job design, safe work, vaccination access and healthy environments.
- Universal support can be combined with targeted pathways.

Programme design cycle

1. Diagnose needs
2. Prioritise outcomes
3. Co-design interventions
4. Plan safeguards and delivery
5. Pilot
6. Evaluate and improve

Quality design criteria

- Relevant: Addresses a material, evidenced need and defined employee group.
- Accessible: Works across shift, location, language, disability, caregiving and device constraints.
- Safe: Protects privacy, clinical boundaries, voluntariness and escalation.
- Evaluable: Has baseline, logic, owner, measures and review timing.

Policy-to-practice chain

1. Policy intent
2. Role and decision right
3. Daily routine
4. Tool or resource
5. Evidence of use
6. Review and improvement

Manager playbook essentials

- Notice: Recognise work-related changes without diagnosing.
- Ask: Choose a private moment and use open, respectful questions.
- Act: Adjust work within authority and connect the employee to support.
- Escalate: Follow safety, HR and emergency routes for risk or complexity.

Individual resilience and organisational resilience

Perspective A

- Self-efficacy, optimism, recovery skills and social support can be protective.
- Practice, coaching and reflection can build usable coping resources.
- Participation should not imply blame for harmful work conditions.

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Perspective B

- Leaders shape workload, autonomy, trust, purpose and access to help.
- Teams create norms for speaking up, support and recovery.
- Organisations remove systemic demands and learn from workforce evidence.

Resilience ecosystem

- Leadership: Visible priorities, psychologically safe behaviour and accountability.
- Job design: Manage demand, control, staffing, role clarity and recovery time.
- Social resources: Peer connection, manager support and trusted communities.
- Professional support: Clear, confidential and timely access to qualified help.

Implementation workstreams

1. Sponsor and governance
2. Service and vendor readiness
3. Manager enablement
4. Communication and access
5. Data safeguards
6. Monitoring and issue resolution

Roles and ownership

- Sponsor: Sets priority, resources and accountability for outcomes.
- Programme owner: Coordinates plan, risks, vendors, data and review.
- Managers: Apply work practices, enable access and respond safely.
- Employees and representatives: Co-design, participate voluntarily and provide feedback.

Participation barriers

- Awareness: Employees do not know what exists or why it is relevant.
- Time and access: Shifts, workload, caregiving, location or technology block use.
- Trust: Privacy, stigma or employment consequences feel uncertain.
- Perceived value: The offer does not match needs, culture or employee experience.

Ethical engagement

1. Explain the need
2. Offer meaningful choices
3. Protect non-participants
4. Remove access barriers
5. Use supportive incentives
6. Monitor equity and unintended effects

Partner and vendor governance

- Capability: Qualified staff, service standards, escalation and clinical governance.
- Privacy: Purpose, data flows, security, retention, transfer and breach duties.
- Access: Hours, languages, channels, accessibility and capacity.
- Evidence: Safe aggregates, service quality, outcomes and corrective action.

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Budget and value

Perspective A

- Budget for design, communication, manager time, access support and evaluation—not just vendor fees.
- Use a benefits logic and realistic time horizon.
- State uncertainty and avoid promising guaranteed ROI.

Perspective B

- Prioritise statutory and safety requirements first.
- Compare reach, equity, quality and outcome with cost.
- Scale only after a pilot demonstrates safe, relevant value.

Pilot-to-scale gates

1. Readiness
2. Safe launch
3. Access and reach
4. Experience and quality
5. Early outcome signal
6. Decision to adapt, scale or stop

Implementation is the programme.

Key point: Implementation is the programme. Policy, vendor and launch event matter only when daily work, access and follow-through change.

Activity 4 — Resilience Ecosystem After a Service Surge

Outcome: LO2. Redesign job demands and resources while adding proportionate resilience support.

Goal and Deliverable

Respond to fatigue and strain in a high-demand team without shifting responsibility onto employees to become tougher.

- Deliverable: Resilience ecosystem map and a 90-day multi-level implementation plan

Real-World Scenario

CIPD's evidence review treats resilience as influenced by individual and contextual factors and cautions against placing the onus on employees to withstand harmful conditions. Protective factors include self-efficacy, positive affect, social support and leader relationships; effective development tends to be structured and experiential rather than a one-off lecture.

CivicConnect, a fictional public-facing service centre, experienced a four-month demand surge. Employees report unpredictable overtime, abusive callers, low control and slow escalation. Management initially proposes a one-hour resilience webinar.

Source: https://www.cipd.org/globalassets/media/knowledge/knowledge-hub/evidence-reviews/employee-resilience-discussion-report_tcm18-91717.pdf

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Training Assumptions and Guardrails

- CivicConnect and all employee evidence are fictional training inputs.
- There is no evidence that a one-hour webinar alone will address the primary work demands.
- The operating budget allows changes to rosters, escalation, peer support and a targeted development programme.
- Any clinical support must be delivered by appropriately qualified providers.

Scenario Questions

Question 1: Which demands are preventable, reducible or unavoidable?

Question 2: Which resources are missing at individual, team and organisational levels?

Question 3: What sequence balances immediate relief with sustainable redesign?

Question 4: How will leaders know whether the change improves recovery and service?

Detailed Step-by-Step Activity

Step 1 — Map the employee journey

Trace a typical shift from preparation to recovery. Mark workload peaks, emotional demands, control, breaks, support and handoffs.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Classify demands

Separate hindrance demands to remove, challenge demands to resource and unavoidable exposure to mitigate. Cite employee evidence for each.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Map existing resources

Assess role clarity, autonomy, staffing, skills, supervisor support, peer support, recovery time and professional help.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Design immediate controls

Choose actions for the next two weeks: workload triage, escalation, break protection, supervisor check-ins and post-incident support.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Design systemic changes

Revise staffing, scheduling, abusive-caller policy, decision authority, queue design and manager accountability.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Add capability support

Specify a structured, experiential resilience component that complements work redesign. Include practice, peer learning, coaching and referral boundaries.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 7 — Plan implementation

Assign sponsor, owner, resources, consultation, communications, milestones, risks and feedback routes for 30/60/90 days.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Set evaluation rules

Choose demand, resource, experience, service and safety measures. Define equity checks and a management review date.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: The plan removes or reduces at least two preventable demands, adds team and organisational resources, treats individual training as complementary, includes safe escalation and has balanced employee/service measures.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 5 — Whole-Person Wellness Programme Portfolio

Outcome: LO2. Design an inclusive portfolio for distinct employee needs.

Goal and Deliverable

Convert a broad wellness ambition into a coherent prevention-to-recovery portfolio with access, ownership and evidence.

- Deliverable: Whole-person programme architecture with component logic models and priorities

Real-World Scenario

The published VHA Whole Health model emphasises whole-person goals and values, health coaching, wellbeing programmes and clinical care within a wider system. It illustrates how an ecosystem can orient support around what matters to the person rather than a fragmented menu of services.

HarbourView Hospitality, a fictional hotel group, wants one portfolio for 900 shift, office, food-service and facilities employees. Current offers are an annual health screen and ad-hoc fitness challenges with low shift-worker participation.

Source: https://pmc.ncbi.nlm.nih.gov/articles/PMC8168167/

Training Assumptions and Guardrails

- HarbourView, workforce data and current service performance are fictional.
- The organisation can use HPB resources where eligible but must validate current programme terms before commitment.
- Employee groups differ in schedule, physical demand, language, financial concerns and digital access.
- Health screening results will not be shared with line managers.

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Scenario Questions

Question 1: Which shared and targeted needs should the portfolio address?

Question 2: How will prevention, early help, support and recovery connect?

Question 3: What makes each pathway accessible and trustworthy for shift employees?

Question 4: Which components should be piloted, scaled, partnered or stopped?

Detailed Step-by-Step Activity

Step 1 — Synthesize needs

Use the supplied personas and data to identify shared conditions and distinct barriers. Label evidence confidence and gaps.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Write portfolio principles

Agree voluntariness, privacy, inclusion, clinical boundaries, paid access, employee voice and job-design responsibilities.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Map the current offer

Place existing services across prevention, promotion, early help, treatment and recovery. Identify duplication and missing pathways.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Generate options

Design physical, mental, financial and social components plus work-design changes. Link every option to a stated need.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Build logic models

For each shortlisted component, map inputs, activity, reach, experience, condition or behaviour change and intended outcome.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Design access

Specify shift timing, languages, disability access, device alternatives, manager release time, confidentiality and navigation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Prioritise the portfolio

Score need, equity, evidence, feasibility, risk, cost and measurability. Choose pilot, scale, partner or stop.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 8 — Create governance

Assign sponsor, programme owner, provider responsibilities, employee advisory input, safeguards and quarterly review.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: Every component links to an evidenced need, the portfolio covers work conditions plus support, access is designed for distinct groups, clinical/privacy boundaries are explicit and each component has an owner and review measure.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 6 — Mental-Wellbeing Policy-to-Practice Playbook

Outcome: LO2. Translate policy into safe day-to-day manager and team practices.

Goal and Deliverable

Turn a generic mental-wellbeing policy into observable routines, decision rights, scripts and escalation paths.

- Deliverable: Policy-to-practice manager playbook and implementation evidence plan

Real-World Scenario

Singapore's tripartite guidance encourages employers to strengthen mental wellbeing through organisation-, team- and individual-level practices, including leadership, supportive managers, work-life harmony, access to help and appropriate confidentiality.

EastLine Distribution, a fictional employer, has a two-page wellbeing policy but inconsistent practice. One manager demands clinical details, another ignores repeated overtime, and employees do not understand when HR or emergency support becomes involved.

Source: https://www.tal.sg/tafep/employment-practices/mental-health

Training Assumptions and Guardrails

- All characters and events are fictional learning scenarios.
- Managers are not clinicians and must not diagnose or provide therapy.
- The organisation has HR, an EAP and a documented emergency escalation route.
- Work adjustments depend on role and risk but should use only necessary information.

Scenario Questions

Question 1: Which policy statements need a concrete routine, tool or decision right?

Question 2: What can managers ask, record, decide and escalate?

Question 3: How should the playbook handle immediate safety concerns and ordinary work support differently?

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Question 4: What evidence demonstrates implementation beyond attendance at training?

Detailed Step-by-Step Activity

Step 1 — Extract policy intent

Highlight each promise, duty, boundary and escalation requirement. Rewrite vague verbs such as support or promote as observable outcomes.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Map employee moments

Identify normal check-ins, visible change, request for help, work adjustment, conflict, absence, return-to-work and immediate safety concern.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Assign decision rights

For every moment, state employee, manager, HR, safety and provider roles; identify what is outside manager authority.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Write conversation guides

Create short Notice–Ask–Listen–Act–Follow-up prompts using observable facts, privacy-respecting questions and no diagnosis.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Define documentation

Specify the minimum record, access, storage and retention. Separate operational adjustment information from clinical details.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Build escalation routes

Distinguish routine referral, complex case consultation, urgent risk and emergency response. Include after-hours paths and handoff confirmation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Plan capability rollout

Use demonstration, practice scenarios, feedback and supervisor coaching. Add resources, communications and employee-facing expectations.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Verify implementation

Select audits, employee feedback, manager confidence, response-time and case-quality evidence; assign corrective action and review.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Acceptance Test

Pass condition: The playbook covers routine through emergency situations, limits collection and diagnosis, names decision rights and handoffs, provides usable scripts and defines evidence of practice quality rather than training completion alone.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 7 — Participation and Partnership Recovery Plan

Outcome: LO2. Remove participation barriers and govern delivery partners for a sustainable programme.

Goal and Deliverable

Diagnose low engagement in an existing programme and redesign access, communication, incentives and vendor governance.

- Deliverable: Participation recovery plan, vendor scorecard and budget-backed experiment

Real-World Scenario

The NCBI review highlights multilevel leadership, organisational alignment, programme scope and quality, accessibility, partnerships and communications as core elements, while identifying awareness, time, manager support, access, privacy and perceived benefit as participation barriers.

SkyGarden Retail, a fictional 1,100-person business, pays for an EAP and wellness portal. Registration is 44%, first use 12%, repeat use 3%, and night-shift uptake is below 1%. Leaders propose a prize draw for registrations.

Source: https://www.ncbi.nlm.nih.gov/books/NBK201567/

Training Assumptions and Guardrails

- All funnel, budget, vendor and employee-comment data are fictional.
- Registration is not evidence of benefit and may be a vanity metric.
- The vendor can provide privacy-safe aggregates but the current contract lacks response-time and accessibility standards.
- A S$35,000 budget can be reallocated across access, communication, manager release time and service capacity.

Scenario Questions

Question 1: Where does the participation funnel fail for each workforce group?

Question 2: Which barriers are awareness, access, trust, relevance, manager or service-quality problems?

Question 3: Would the proposed incentive support or distort the intended behaviour?

Question 4: What partner standards and experiment would restore sustainable participation?

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Detailed Step-by-Step Activity

Step 1 — Analyse the funnel

Calculate eligible-to-aware, aware-to-register, register-to-first-use and first-to-repeat conversion by group. Apply small-cell suppression.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Code employee voice

Group comments into awareness, time, access, trust, relevance, manager and service experience. Preserve contradictory evidence.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Find root causes

Use a barrier map to distinguish communication symptoms from work-design, service-capacity, privacy and vendor causes.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Review the incentive

Test voluntariness, fairness, behaviour targeted, privacy and sustainability. Redesign or reject the prize draw with reasons.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Strengthen the partnership

Add service standards for response, quality, escalation, accessibility, safe aggregates, security, breach handling and corrective action.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Reallocate budget

Fund changes across paid access time, shift coverage, trusted communication, language/accessibility, service capacity and evaluation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Design a recovery experiment

State target group, barrier, change, baseline, measure, duration, safeguard and success/stop rule.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Set governance

Assign owner, vendor forum, issue route, monthly evidence review and decision date for adapt, scale or exit.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: The plan diagnoses funnel loss by group, addresses at least three root-cause categories, avoids coercive incentives, sets vendor quality/privacy standards and defines a measurable

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experiment with owner and stop rule.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Topic 3 — Communication, Feedback and Improvement

Focus: benefits · audience · channels · trust · feedback · metrics · management review.

Key Concepts

- Benefit: Describe a credible employee and organisational value proposition without overpromising.
- Communication: Match message, messenger, channel, timing and call to action to each audience.
- Feedback: Combine safe quantitative and qualitative evidence; make it easy to speak and see a response.
- Improvement: Use a logic model, equity checks and management decisions to adapt or stop programmes.

Trust is part of programme access.

Key point: Trust is part of programme access. People will not use support they believe could expose, label or disadvantage them.

Benefits for employees and the organisation

Perspective A

- Easier access to relevant support and healthier work conditions.
- Greater capability, energy, connection and confidence to seek help.
- Fairer access across locations, shifts and life circumstances.

Perspective B

- Potential improvements in absence, retention, engagement and sustainable performance.
- Better risk visibility and manager capability.
- Benefits depend on context and implementation; association is not proof of causation.

Communication design

1. Define behaviour or decision
2. Segment audience
3. Find barrier and motivation
4. Craft message and proof
5. Choose channel and messenger
6. Measure response and adapt

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Audience differences

- Desk-based and hybrid: Digital reach is high, but attention and isolation may be barriers.
- Frontline and shift: Supervisor briefings, roster timing and paid access may matter more than email.
- Managers: Need role clarity, scripts, boundaries and escalation—not only awareness.
- Vulnerable or underserved groups: Require trusted messengers, accessible formats and stronger privacy assurance.

Message architecture

- Why: Name the employee need and intended benefit in plain language.
- What: Explain the offer, boundaries and what it does not do.
- How: Give a simple, accessible action and alternative path.
- Trust: State privacy, voluntariness, provider and help or complaint route.

Communication channels

- Digital: Email, intranet, app, Teams, SMS and digital signage.
- Human: Managers, champions, HR, union or employee representatives and peers.
- Physical: Toolbox talks, posters, payslip inserts and onsite events.
- Service moment: Onboarding, return-to-work, benefits enrolment and manager check-ins.

Consistency and localisation

Perspective A

- Keep eligibility, privacy and access promises consistent.
- Use a single approved source for dates, contacts and service boundaries.
- Train messengers before launch.

Perspective B

- Adapt examples, language, timing and channel to the audience.
- Use culturally relevant and accessible formats.
- Test comprehension, not merely message delivery.

Employee feedback system

1. Ask a focused question
2. Offer safe channels
3. Protect identity
4. Analyse themes and gaps
5. Decide and assign
6. Close the loop

Feedback methods

- Survey: Scalable trends and segments; vulnerable to low response and design bias.
- Focus group: Rich shared experience; power dynamics and confidentiality need facilitation.
- Interview: Deep individual context; slower and harder to anonymise.
- Programme and service data: Behavioural evidence; may not explain why people did or did not engage.

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Additional listening channels

- Social listening: Use public or authorised data, clear purpose and ethical boundaries.
- Exit and stay interviews: Reveal experience and retention factors; avoid treating departure as the only safe time to speak.
- Suggestion and speak-up routes: Enable ongoing issues with acknowledgement and escalation.
- Manager observations: Useful operational signal, but not a proxy for confidential employee voice.

Anonymous and confidential

Perspective A

- Anonymous means identity is not collected or reasonably recoverable.
- Small teams, free text and metadata can defeat anonymity.
- Set minimum reporting cells and redact identifiers.

Perspective B

- Confidential means authorised people can know identity under defined limits.
- Explain who sees what and when escalation may occur.
- Do not promise anonymity when the channel is only confidential.

Programme logic model

1. Inputs
2. Activities
3. Reach and participation
4. Experience and capability
5. Behaviour or condition change
6. Wellbeing and organisational outcomes

Balanced measures

- Reach: Eligible employees, awareness and access by relevant group.
- Participation: Uptake, repeat use, completion and drop-off.
- Experience: Trust, relevance, accessibility, quality and psychological safety.
- Outcome: Work conditions, wellbeing indicators and operational outcomes over time.

Leading and lagging indicators

Perspective A

- Leading: manager capability, access, response time, workload controls and participation equity.
- They show conditions that can influence later outcomes.
- Use them for early correction.

Perspective B

- Lagging: absence, turnover, claims, incidents and sustained wellbeing outcomes.
- They are influenced by many factors and often change slowly.
- Avoid attributing change to one programme without suitable evidence.

Evaluation threats

- Selection bias: Participants may differ from non-participants before the programme.
- Confounding: Workload, leadership, seasonality or policy changes influence outcomes.

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- Small numbers: Percentages may fluctuate and create privacy risk.
- Vanity metrics: Registrations and clicks do not prove benefit or safe implementation.

Management review

1. Review purpose and safeguards
2. Compare target with evidence
3. Check equity and unintended effects
4. Identify causes
5. Choose scale, adapt or stop
6. Assign and close actions

Decision-ready report

- Decision: What approval, resource, risk acceptance or change is required?
- Evidence: What is known, how reliable is it and what remains uncertain?
- Equity and risk: Who benefits, who is missing and what harm may occur?
- Action: Owner, due date, measure, communication and next review trigger.

Feedback earns trust only when people can see what changed.

Key point: Feedback earns trust only when people can see what changed. Close the loop, explain constraints and keep measuring the employee experience.

Activity 8 — Inclusive Multi-Channel Wellbeing Campaign

Outcome: LO3. Communicate credible programme benefits through appropriate techniques and channels.

Goal and Deliverable

Design a trust-centred campaign that reaches office, frontline and mobile employees without overclaiming outcomes.

- Deliverable: Audience-message-channel matrix and six-week campaign plan

Real-World Scenario

Gallup's research treats career wellbeing as foundational and describes employee wellbeing as connected with engagement, absence, performance and retention. These relationships can guide a value proposition but do not prove that a single campaign or programme causes each outcome.

MetroCare Facilities, a fictional employer, is launching a whole-person support hub. Past email-only campaigns reached office staff but missed cleaners, technicians and overnight teams. Employees worry that managers can see EAP usage.

Source: https://www.gallup.com/workplace/404105/importance-of-employee-wellbeing.aspx

Training Assumptions and Guardrails

- Personas, channel reach, trust scores and message-test results are fictional.
- EAP individual use is confidential to the provider, subject to stated safety and legal boundaries.
- The campaign has six weeks and a S$18,000 budget.
- No message may promise improved health, productivity or promotion outcomes to an individual participant.

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Scenario Questions

Question 1: What behaviour or decision should each audience take?

Question 2: Which benefit, barrier, messenger and channel combination fits each group?

Question 3: How will the campaign explain privacy and voluntariness accurately?

Question 4: Which measures distinguish delivery, comprehension, trust and action?

Detailed Step-by-Step Activity

Step 1 — Define campaign outcomes

Set one awareness, one comprehension and one access behaviour per priority audience. Avoid vague goals such as raise awareness.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Segment by access need

Use role, schedule, location, language and channel access rather than unsupported demographic stereotypes.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Map barriers and motivations

For each audience, record time, trust, relevance, manager, digital and service barriers plus a credible employee benefit.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Write the message architecture

Create Why–What–How–Trust copy with accurate eligibility, provider, privacy boundary, action and alternative route.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Select channels and messengers

Combine digital, physical, manager, peer and service-moment channels. Add paid time or roster access where communication alone cannot solve reach.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Prototype and test

Run comprehension and trust testing with diverse employees. Record misinterpretations and revise before launch.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Plan cadence

Sequence pre-launch trust, launch action, manager reinforcement and reminder messages without fatigue. Keep details consistent across channels.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Step 8 — Measure and adapt

Track reach, comprehension, trust, access attempts and equitable uptake. Define weekly corrections and a post-campaign review.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: Every priority audience has a defined action, credible benefit, trusted messenger and accessible channel; privacy is accurate; overclaiming is absent; and measures cover reach, comprehension, trust, action and equity.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 9 — Employee Feedback and Listening System

Outcome: LO3. Gather safe, representative employee feedback using complementary methods.

Goal and Deliverable

Design a feedback system that combines surveys, focus groups, interviews and programme data while protecting identity and closing the loop.

- Deliverable: Multi-method listening plan, draft instruments and close-the-loop protocol

Real-World Scenario

The Center for Workplace Mental Health describes organisational resilience through leadership, training, a culture of empowerment, purpose, trust and accountability, supportive work environments, flexibility and access to support. Employee voice is needed to understand whether those conditions are experienced in practice.

RiverTech Digital, a fictional hybrid employer, wants quarterly feedback after a wellbeing launch. Employees distrust a survey that collects team, age, disability, free text and email. Managers want raw comments by team.

Source: https://workplacementalhealth.org/mental-health-topics/resilience

Training Assumptions and Guardrails

- All sample responses and organisational details are fictional.
- Reporting groups must meet a minimum threshold and free text needs disclosure review.
- An independent research administrator is available for confidential interviews.
- Immediate safety disclosures require a defined, transparent escalation route.

Scenario Questions

Question 1: Which question requires survey, focus group, interview or service data?

Question 2: What data should not be collected, and what reporting threshold is safe?

Question 3: How will employees understand anonymity, confidentiality and escalation?

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Question 4: How will management acknowledge, decide and close the feedback loop?

Detailed Step-by-Step Activity

Step 1 — Define listening questions

Write a small set of decisions the feedback must inform: access, trust, quality, work conditions or outcome. Remove curiosity-only collection.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Choose complementary methods

Match survey, focus group, interview and programme data to each question. Record strengths, bias, burden and privacy risk.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Design sampling

Identify who must be heard, recruitment route, participation barriers, minimum cell sizes and actions for underrepresented groups.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Draft instruments

Use neutral, single-focus questions, appropriate scales and optional free text. Avoid clinical screening unless qualified governance exists.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Write the privacy notice

Explain purpose, voluntariness, data fields, administrator, recipients, aggregation, retention, withdrawal limits and safety escalation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Create the analysis protocol

Predefine denominators, suppression, coding, contradictory evidence, uncertainty and free-text redaction. Prohibit manager access to raw comments.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Build the response workflow

Set acknowledgement, triage, urgent escalation, decision owner, action tracking and employee communication timelines.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Pilot and refine

Run cognitive testing and a small pilot. Check comprehension, completion, disclosure risk and whether results can support the intended decision.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

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Acceptance Test

Pass condition: The system collects only decision-relevant data, uses complementary methods, protects small groups and free text, accurately distinguishes anonymity/confidentiality, includes safety escalation and commits to visible response timelines.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Activity 10 — Wellbeing Programme Management Review

Outcome: LO3. Assess programme effectiveness and recommend scale, adaptation or stop.

Goal and Deliverable

Turn reach, experience, work-condition, outcome and cost evidence into a decision-ready management review.

- Deliverable: Balanced programme dashboard and management decision paper

Real-World Scenario

McKinsey's workplace research presents wellbeing as a leadership priority and highlights organisational enablers such as psychological safety, purpose and adaptable support. The source supports a systems perspective; the fictional programme dataset must be evaluated on its own quality and cannot inherit causal conclusions.

ApexFin Services, a fictional 760-person employer, piloted manager training, protected focus time, EAP navigation and peer mentoring for 12 months. Participation rose, but absence and turnover moved differently across units. Leaders ask whether to scale nationally.

Source: https://www.mckinsey.com/featured-insights/mckinsey-guide-to-navigating-the-new-world-of-work/thriving-at-work-building-resilience-wellness-and-motivation

Training Assumptions and Guardrails

- All programme, outcome, cost, baseline and comparison data are fictional.
- Business units were not randomly assigned; workload and leadership changes occurred during the pilot.
- Small groups and sensitive service data must be aggregated before management reporting.
- The decision options are scale, adapt and extend pilot, pause, or stop—not simply pass/fail.

Scenario Questions

Question 1: Did delivery reach the intended groups safely and equitably?

Question 2: What evidence supports changes in experience, work conditions and outcomes?

Question 3: Which alternative explanations and data-quality weaknesses limit confidence?

Question 4: What should management scale, adapt, pause or stop, and what is the next review trigger?

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Detailed Step-by-Step Activity

Step 1 — Reconstruct the logic model

Map inputs, activities, reach, experience, capability or condition change and outcomes. Mark where the causal chain is unsupported.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 2 — Validate the dataset

Check definitions, periods, denominators, missingness, duplicates, group size, baseline comparability and privacy suppression.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 3 — Assess implementation

Compare planned with delivered dose, manager participation, service quality, access and safeguards. Explain material deviations.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 4 — Analyse reach and equity

Calculate eligibility, awareness, uptake, repeat use and completion by safe segment. Identify who was missed or burdened.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 5 — Evaluate outcomes cautiously

Compare baseline and trend for experience, work conditions, wellbeing and operational measures. State confounders and avoid unsupported causation.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 6 — Assess value and risk

Compare cost with reach, quality and credible outcome signals. Review incidents, complaints, vendor risk and unintended effects.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 7 — Make component decisions

For each intervention, recommend scale, adapt, pause or stop with evidence confidence, condition and owner.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Step 8 — Run management review

Present decision, evidence, equity, uncertainty, resource need, action, communication and next review trigger; record management acceptance.

Checkpoint: Record the evidence, calculation or decision produced before moving to the next step.

Acceptance Test

Pass condition: The review validates data, follows the logic model, separates implementation from

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outcome, analyses equity, names confounders, avoids causal overclaiming and gives component-level decisions with owners and review triggers.

Reflection and Debrief

1. Which assumption changed your recommendation most, and why?
2. What additional evidence would you request?
3. Which stakeholder owns the next action?
4. Which review trigger or future exercise should follow?

Sources and Further Reading

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Gallup — The importance of employee wellbeing: https://www.gallup.com/workplace/404105/importance-of-employee-wellbeing.aspx

CIPD — Employee resilience evidence review: https://www.cipd.org/globalassets/media/knowledge/knowledge-hub/evidence-reviews/employee-resilience-discussion-report_tcm18-91717.pdf

Center for Workplace Mental Health — Resilience: https://workplacementalhealth.org/mental-health-topics/resilience

W Talent Solutions — Wellness in the workplace: https://wtalentsolutions.com/wellness-in-the-workplace/

McKinsey — Thriving at work: https://www.mckinsey.com/featured-insights/mckinsey-guide-to-navigating-the-new-world-of-work/thriving-at-work-building-resilience-wellness-and-motivation

PMC — Resilience ecosystem in high-stress occupations: https://pmc.ncbi.nlm.nih.gov/articles/PMC13206323/

Outback Team Building — Corporate wellness and resiliency: https://www.outbackteambuilding.com/blog/corporate-wellness-and-resiliency/

PMC — Whole Health System of Care: https://pmc.ncbi.nlm.nih.gov/articles/PMC8168167/

PDPC Singapore — Data protection obligations: https://www.pdpc.gov.sg/overview-of-pdpa/the-legislation/personal-data-protection-act/data-protection-obligations

TAFEP — Tripartite advisory on mental well-being at workplaces: https://www.tal.sg/tafep/employment-practices/mental-health

HPB — Workplace health programmes: https://www.hpb.gov.sg/workplace

MOM — Sick leave eligibility and entitlement: https://www.mom.gov.sg/employment-practices/leave/sick-leave/eligibility-and-entitlement

Healthier SG — What is Healthier SG?: https://www.healthiersg.gov.sg/about/what-is-healthier-sg/

Tertiary Courses — official course page: https://www.tertiarycourses.com.sg/improve-employee-wellness-and-work-resilience.html

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Glossary

- EAP: Employee Assistance Programme: confidential support and referral services offered through an employer.
- Job demands: Aspects of work requiring sustained effort and associated with physical or psychological cost.
- Job resources: Aspects of work that help achieve goals, reduce demands or support growth and recovery.
- Psychological safety: A shared belief that interpersonal risk-taking, speaking up and asking for help are safe.
- Aggregate data: Information combined across people; small groups or detailed fields can still create re-identification risk.
- Leading indicator: A condition or activity expected to influence future outcomes.
- Lagging indicator: An outcome observed after conditions and interventions have acted.
- Logic model: A causal map connecting inputs, activities, reach, experience and intended outcomes.

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